

**Patient Name :** Vikas Gandhi  
**Age / Sex :** 52 Y / M  
**Referred By :** Dr. ASHISH TYAGI  
**Centre :** GHAZIABAD

**Lab No :** GZB25062838  
**Registration On :** 15-Jun-25 08:01  
**Patient ID :** UKKD.0000138009

**Urine R/M** **Urine Sample**

**Accession No:** CP00672523 **Collected On:** 15-Jun-25 08:01 **Received On:** 16-Jun-25 11:59 **Approved On:** 16-Jun-25 15:47

| Observation                     | Result      | Unit | Biological Ref. Interval | Method                     |
|---------------------------------|-------------|------|--------------------------|----------------------------|
| <b>Physical Examination</b>     |             |      |                          |                            |
| Urine Quantity                  | 7.5         | mL   | 7 - 8                    | Physical Examination       |
| Urine Colour                    | Pale Yellow |      | Pale Yellow              | Physical Examination       |
| Urinary Transparency            | Clear       |      | Clear                    | Physical Examination       |
| <b>Biochemical Examination</b>  |             |      |                          |                            |
| Urinary pH                      | 6.0         | pH   | 6.0 - 8.0 pH             | bromothymol blue           |
| Urinary Specific Gravity        | 1.025       |      | 1.005 - 1.030            | Ethylene glycol-bis t.a.a. |
| Urinary Protein                 | Negative    |      | Negative                 | Tetrachlorophenol          |
| Urinary Glucose                 | Negative    |      | Negative                 | glucose-oxidase-peroxidase |
| Urinary Ketones                 | Negative    |      | Negative                 | Sodium Nitroprusside       |
| Urobilinogen                    | Normal      |      | Normal                   | Methoxybenzene Diazonium   |
| Urine Bilirubin                 | Negative    |      | Negative                 | Dichlorobenzene-diazonium  |
| Urinary Nitrites                | Negative    |      | Negative                 | hydroxy                    |
| Blood [In Urine]                | Negative    |      | Negative                 | Tetramethylbenzidine       |
| Leukocyte esterase              | 1+          |      | Negative                 | indoxyl-ester-diazonium    |
| <b>Microscopic Examination</b>  |             |      |                          |                            |
| Pus Cells [In Urine]            | 10-15       | /HPF | 1 - 2 /HPF               | Flow Micro Imaging         |
| Epithelial Cells (Squamous)     | 1-2         | /HPF | 0-2/HPF                  | Flow Micro Imaging         |
| Epithelial Cells (Non-Squamous) | NIL         | /HPF | 0-2/HPF                  | Flow Micro Imaging         |
| Urinary RBC                     | NIL         | /HPF | NIL /HPF                 | Flow Micro Imaging         |
| Hyaline Casts                   | NIL         | /LPF | 0-2/LPF                  | Flow Micro Imaging         |
| Pathological Casts              | NIL         | /LPF | 0-1/LPF                  | Flow Micro Imaging         |
| Yeast Cells                     | NIL         | /HPF | 0-1/HPF                  | Flow Micro Imaging         |
| Crystals                        | NIL         | /HPF | NIL/HPF                  | Flow Micro Imaging         |
| Other Morphology                | NIL         |      | NIL                      | Microscopy                 |

**Remarks:**

- Sample Quantity is observed after transfer to a Urinalysis Vacutainer Tube for preservation of sample.
- Note for Female Patients:** If the urine is collected during menstruation, red cells may be present in the urine.
- Microscopy:** Microscopy may have supplemented automated measurements, wherever necessary.

**Advise:** Please correlate results clinically.

*Amita Gupta*  
**Dr. Amita Gupta**  
 Consultant – Pathologist  
 M.B.B.S., M.D. (Pathology)  
 DMC Reg. No.: 91678

In case of any unexpected or alarming results, please contact us immediately for re-confirmation, clarifications, and rectifications, if needed.

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## Ultrasound KUB

Approved On: 15-Jun-25 11:29

### ULTRASOUND KUB

**Right kidney** is normal in size (~ 93 x 44 mm) shape and echotexture. Corticomedullary differentiation is maintained. No evidence of hydronephrosis is seen. **A calculus measuring ~ 5.5 mm is seen at mid pole.**

**Left kidney** is normal in size (~ 96 x 50 mm) shape and echotexture. Corticomedullary differentiation is maintained. No evidence of hydronephrosis is seen. **A calculus measuring ~ 5.0 mm is seen at lower pole.**

**Urinary bladder** is seen in distended state. No evidence of calculus or mass lesion is seen. Wall thickness is normal.

**Prevoid volume measures ~ 155 cc. Post void volume measures ~ 52 cc (significant).**

**Prostate appears enlarged in size and measures ~ 44 cc in volume.** No obvious focal lesion is seen.

### IMPRESSION: ULTRASOUND KUB REVEALS:

--> **BILATERAL RENAL CALCULUS.**

--> **GRADE II PROSTATOMEGALY WITH SIGNIFICANT POST VOID RESIDUAL URINE.**

### ADVISED: CLINICAL, URINE R/M & SERUM PSA CORRELATION.

*Investigations have their limitations. Solitary investigations never confirm the final diagnosis of disease. It only helps in diagnosing the disease in correlation to the clinical symptoms. Not meant for medico legal purposes.*



**Dr. Dev Kumar**  
Consultant Radiologist  
M.B.B.S., M.D. (Radio-Diagnosis)  
UPMC Reg. No.: 84832

*In case of any discrepancy due to typing error, kindly get it rectified immediately. This is professional opinion, not a diagnosis.*

### Hb A1c

### EDTA Whole Blood Sample

Accession No: ED01409834 Collected On: 15-Jun-25 08:01 Received On: 15-Jun-25 12:30 Approved On: 15-Jun-25 15:47

| Observation                  | Result | Unit  | Biological Ref. Interval | Method     |
|------------------------------|--------|-------|--------------------------|------------|
| HbA1C                        | 8.0    | %     | 4.8-5.7                  | HPLC       |
| 90 Day Average Blood Glucose | 182.9  | mg/dl | 90 - 120                 | Calculated |

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**Interpretation as per American Diabetes Association (ADA) Guidelines**

| Reference Group | Non diabetic adults $\geq 18$ years | At risk (Prediabetes) | Diagnosing Diabetes | Therapeutic goals or glycemic control |
|-----------------|-------------------------------------|-----------------------|---------------------|---------------------------------------|
| HbA1c in %      | 4.0-5.6                             | 5.7-6.4               | $\geq 6.5$          | $< 7.0$                               |

Note:

- Presence of Hemoglobin variant and /or conditions that affect red cell turnover must be considered particularly when the HbA1c result does not correlate with the patient's blood glucose levels.
- Factors That Interfere With Hba1c Measurement:** Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements
- Factors That Affect Interpretation Of Hba1c Results:** Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, Hbss, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c
- Since HbA1c Reflects long term Fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
- Target goals of  $< 7.0\%$  may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular co-morbid conditions, targeting a goal of  $> 7.0\%$  may not be appropriate.
- Any condition that shortens erythrocyte survival such as sickle cell disease, pregnancy (second and third trimesters), hemodialysis, recent blood loss of transfusion , or erythropoietin will falsely lower HbA1c results regardless of the assay method
- In patients with HbA1c level between 7-8%, Glycemark (1,5 Anhydro-glucitol) test may be done to identify those with more frequent and extreme hyperglycemic excursions.

**Remarks:** Please correlate results with clinical conditions.

  
**Dr. Pankaj Bhimrao Bhagat**  
 Consultant – Pathologist  
 M.B.B.S., M.D. (Pathology)  
 DMC Reg. No.: DMC/R/5792

  
**Dr. Santosh Suman**  
 Consultant – Pathologist  
 M.B.B.S., M.D. (Pathology)  
 DMC Reg. No.: R/11938

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| Urine C/S                       |                                                                           |                                     | Urine Sample                        |         |
|---------------------------------|---------------------------------------------------------------------------|-------------------------------------|-------------------------------------|---------|
| <b>Accession No:</b> CS00156183 | <b>Collected On:</b> 15-Jun-25 08:01                                      | <b>Received On:</b> 16-Jun-25 12:01 | <b>Approved On:</b> 18-Jun-25 12:32 |         |
| Observation                     | Result                                                                    | Unit                                | Biological Ref. Interval            | Method  |
| Culture Result                  | <b>NO GROWTH AFTER 24 - 48 HRS OF INCUBATION AT 37 DEGREES CENTIGRADE</b> |                                     |                                     | CULTURE |

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**Legend:**

- S : Susceptible/Sensitive
- I : Intermediate
- R : Resistance
- SDD : Susceptible Dose Dependent
- MIC : Minimum Inhibitory Concentration. Measured in ug/mL

**Procedure :** Culture/Sensitivity samples are inoculated in Biosafety Type II Cabinet and incubated at 37 degree celsius in BOD Incubator for a period of upto 24-48 Hours hours and observed for growth of pathogenic bacteria. Subsequently, observations of pathogenic growth are further evaluated for identification and antibiotic/antifungal susceptibility testing on Fully Automated Vitek 2C System.

**Detailed Legend:**

- S : Susceptible/Sensitive : It implies that isolates are inhibited by the usually achievable concentrations of anti-microbial agent when dosage recommended to treat the site of infection is used.
- I : Intermediate: It includes isolates with antimicrobial agent MICs that approach usually attainable blood and tissue levels, and for which response rates may be lower than for susceptible isolates. The intermediate category implies clinical efficiency in body sites where the drugs are physiologically concentrated or when a higher than normal dosage of a drug can be used.
- R : Resistance: It implies that isolates are not inhibited by the usually achievable concentrations of the agent with normal dosage schedules, and /or that demonstrate MICs or zone diameters that fall in the range where specific microbial resistance mechanisms (e.g. Beta-lactamases) are likely, clinical efficiency of the agent against the isolate has not been reliable shown in treatment studies.

**Note:** The following conditions can cause a False Negative Culture:

- Patient is on antibiotics. Please repeat culture post therapy.
- Anaerobic bacterial infection.
- Fastidious aerobic bacteria which are not able to grow on routine culture media.
- Besides all these factors, at least in 25-40 % of cases there is no direct correlation between in vivo clinical picture and in vitro culture result

**Clinical Remarks:** The AST results are in vitro and in vivo clinical results may be unreliable depending upon PK - PD of the patient. Clinical correlation is required.

**Advantages of Automated VITEK 2C System:**VITEK 2 systems use Advanced Colorimetry™, an identification technology that enables identification of routine clinical isolates. Advanced Colorimetry provides:

- High discrimination between species Low rate of multiple choice and misidentified species
- The system includes an ADVANCED EXPERT SYSTEM (AES) that analyzes MIC patterns and detects phenotypes for most organisms tested. This helps optimize laboratory efficiency for lean laboratory management. Rapid results allow clinicians to discontinue empiric therapy and prescribe targeted therapy, resulting in improved patient outcomes and enhanced antibiotic stewardship. With its ability to provide accurate "fingerprint" recognition of bacterial resistance mechanisms and phenotypes, the ADVANCED EXPERT SYSTEM (AES) is a critical component of VITEK 2 technology. VITEK 2 technology with the ADVANCED EXPERT SYSTEM offers:
- Knowledge base developed from >100,000 references
- >2,000 described phenotypes
- >20,000 MIC distributions
- >100 resistance mechanisms detected
- >99 organisms
- On average, provides a resulting range of five to seven MIC doubling dilutions per antibiotic
- Extended MIC range to enable low-level resistance detection

**Advise:** Please correlate results with clinical conditions.

**Dr. Ritika Aggarwal**  
Consultant Microbiologist  
M.B.B.S., M.D. (Microbiology)  
DMC Reg. No.: 67545



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## Conditions Of Reporting

- ▶ The report results are for information and interpretation for your referring doctor. Reports are to be correlated with the patient's clinical history.
- ▶ Biological Reference Range/Interval is suggested for your Gender and Age on the basis of available literature. All reference ranges are to be reconsidered by physician's advice for your specific care.
- ▶ This Medical Report is a professional opinion, not a diagnosis.
- ▶ The report will carry the name and age provided at the time of registration. To maintain confidentiality, certain reports may not be e-mailed at the discretion of the management.
- ▶ All the notes and interpretation beneath the pathology result in the report provided are for educational purpose only. It is not intended to be a substitute for physician's consultation.
- ▶ Results of tests may vary from laboratory to laboratory and in some parameters from time to time for the same patients. Test results and reference range may also vary depending on the technology and methodology used. Laboratory test results may also vary depending on the age, sex, time of the day sample has been taken, diet, medication and limitation of modern technology.
- ▶ In case of any unexpected or alarming test results, please contact us immediately for re-confirmation, further discussion, clarifications and rectifications, if needed.
- ▶ In case of any discrepancy due to typing error, kindly get it rectified immediately.
- ▶ Neither HOD or its employees/representatives assume any liability or responsibility for any loss or damage that may be incurred by any person as a result of interpreting the meaning of this report.
- ▶ Test results are not valid for medico legal purposes.
- ▶ In case of any issues or suggestions about your test results, please email us on [quality@houseofdiagnostics.com](mailto:quality@houseofdiagnostics.com)
- ▶ The courts (forums) at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the tests and the results of the tests. Our liability is limited to the amount of investigations booked with us.

DOC#COR20200707

## Facilities Available

### Radiology

- ▶ 3T MRI & 1.5T MRI
- ▶ CT Scan
- ▶ Digital X-Ray
- ▶ Mammography
- ▶ Open / Standing MRI
- ▶ Bone DEXA Scan

### Pathology

- ▶ Biochemistry
- ▶ Immunoassay
- ▶ Hematology
- ▶ Clinical Pathology
- ▶ Serology
- ▶ Microbiology

### Nuclear Medicine

- ▶ **India's First** Simultaneous PET-MRI
- ▶ Whole Body PET/CT Scan
- ▶ DTPA / DMSA Renal Scans
- ▶ Thyroid Scan
- ▶ Whole Body Bone Scan
- ▶ HIDA Scan • Rest MUGA

### Cardiology Investigations

- ▶ ECG (Electrocardiogram)
- ▶ Echocardiography
- ▶ TMT
- ▶ Stress Echocardiography
- ▶ Stress Thallium

### Neurology Investigations

- ▶ EEG - ElectroEncephaloGram
- ▶ EMG - ElectroMyoGraphy
- ▶ NCV - Nerve Conduction Velocity
- ▶ VEP - Visual Evoked Response
- ▶ SSEP

### Dental Imaging

- ▶ CBCT - Cone Beam CT Scan
- ▶ OPG - OrthoPantomoGram

### Other Tests

- ▶ PFT